

Policy on Pain Management

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Clinical Lead (Pain Management) Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Pain is treated as the fifth vital sign — every patient entering the hospital is screened for it, patients with significant pain get a detailed assessment, and pain relief is initiated and adjusted based on how each patient responds.

1. Purpose

This policy explains how «Hospital Name» screens patients for pain, carries out a detailed pain assessment where needed, and initiates and adjusts pain relief based on the patient's response.

This policy does not cover general patient assessment or medication management in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all clinical staff involved in screening, assessing, and treating pain at «Hospital Name».

3. Policy standards

«Hospital Name» treats pain as the fifth vital sign, screening every patient entering the hospital for it. Patients whose pain is a predominant symptom, and all post-operative patients, get a detailed, validated pain assessment. Pain relief — non-pharmacological, medical, surgical, or anaesthetic — is initiated based on assessment, monitored for response, and adjusted accordingly.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not manage pain without written guidance covering how patients are screened, how detailed assessment is carried out, what mitigation techniques are used, and how pain is monitored.
- Do not skip pain screening for any patient entering the hospital, and do not fail to document it in the medical record.
- Do not skip a detailed pain assessment for a patient whose pain is a predominant symptom, or for any post-operative patient — use only a validated, age-appropriate tool, covering intensity, character, frequency, location, and duration and radiation.
- Do not initiate pain relief without monitoring the patient's response and adjusting the approach based on that response.

5. What we do

5.1 Guide pain management with written guidance

Written guidance based on sound clinical practice governs how «Hospital Name» manages pain, covering how patients are screened for pain, how a detailed pain assessment is carried out when needed, the pain mitigation techniques used, and how pain is monitored. Care follows both pain management guidelines and the patient's own goals for managing their pain.

5.2 Screen every patient for pain

Pain is treated as the fifth vital sign. Every patient entering the hospital is screened for it — this could be as simple as a yes/no question during the initial assessment. Screening is documented by the doctor or nurse in the medical record. Where an OPD patient's screening comes back positive, that could point to the need for a more detailed assessment.

5.3 Carry out a detailed assessment where needed

A detailed pain assessment is done when pain is the predominant symptom, or one of the main symptoms — for example, cancer pain, neuralgia, or arthralgia. This does not include chest pain from angina, or pain with a physiological cause like labour pain. Regardless of symptom presentation, a detailed assessment is also done for every post-operative patient.

A validated tool — appropriate to the patient type, whether neonate, paediatric, adult, or a patient on a ventilator — is used for this assessment. It covers the intensity of the pain (using an age-appropriate validated pain rating scale), its character, frequency, location, and duration and any referral or radiation. The assessment is done objectively, so it can be reliably compared at reassessment.

5.4 Initiate and adjust pain relief based on response

Based on the assessment and the patient's underlying condition, pain alleviation methods or medication are started. «Hospital Name»'s approach to pain management spans non-pharmacological, medical, surgical, and anaesthetic techniques, guided by the WHO pain ladder scale. The patient is monitored for how they respond, and measures or medication are adjusted accordingly. Where «Hospital Name» doesn't have facilities for further pain management, it could refer the patient to a specialist pain-management centre.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced.
Clinical Lead (Pain Management)	Owns day-to-day implementation; maintains written guidance; ensures validated assessment tools are in place and appropriate to patient type; ensures monitoring and adjustment processes are followed.
Treating doctors	Screen patients for pain; initiate detailed assessments for patients with predominant pain symptoms and for all post-operative patients; initiate, monitor, and titrate pain relief; document in the medical record.
Nursing staff	Screen patients for pain on admission and document in the medical record; participate in pain assessment; monitor patient response to pain relief; escalate uncontrolled pain.
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.

7. Quality monitoring

Monitoring area	What is monitored
Written guidance	Written guidance covering four areas — screening, detailed assessment, mitigation techniques, monitoring — in place, current, and followed
Pain screening	All patients entering the hospital screened for pain; documented by doctor or nurse in the medical record
Detailed pain assessment — trigger 1	Detailed assessment conducted for patients with pain as a predominant or main symptom, excluding chest pain from angina and physiological pain (labour); validated, age-appropriate tool used

Detailed pain assessment — trigger 2	Detailed assessment conducted for all post-operative patients; validated, age-appropriate tool used
Assessment content	Assessment covers all five elements: intensity (using age-appropriate validated pain rating scale), character, frequency, location, and duration and referral/radiation
Pain relief — initiation and adjustment	Pain alleviation initiated after assessment; patient monitored for response; measures and medication adjusted accordingly; four technique categories available per WHO pain ladder: non-pharmacological, medical, surgical, anaesthetic

8. Training and staff acknowledgement

All clinical staff shall be familiar with the requirements of this policy, including: pain as the fifth vital sign and universal entry screening; the two separate detailed-assessment triggers (predominant symptom AND all post-operative patients); the exclusions (chest pain from angina; pain with a physiological cause such as labour pain); the five assessment content elements (intensity, character, frequency, location, duration and referral/radiation); and the four pain management technique categories per the WHO pain ladder scale (non-pharmacological, medical, surgical, anaesthetic).

I have read the Policy on Pain Management of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to all treating doctors, nurses, and clinical staff involved in patient care at «Hospital Name».

10. Abbreviations

Abbreviation	Meaning
COP	Care of Patients (NABH Hospitals chapter)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OPD	Outpatient Department
WHO	World Health Organization

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.17.a	Commitment*	Section 5.1 addresses written guidance based on sound clinical practice, covering four mandatory content areas: how patients are screened for pain, how a detailed pain assessment is carried out when needed, what pain mitigation techniques are used, and how pain is monitored. Care follows both pain management guidelines and the patient's own goals.
COP.17.b	Commitment	Section 5.2 addresses pain as the fifth vital sign — stated explicitly. Every patient entering the hospital is screened for pain. Screening is documented by the doctor or nurse in the medical record.
COP.17.c	Commitment	Section 5.3 addresses detailed pain assessment under two separate triggers: (1) pain is the predominant symptom or one of the main symptoms (examples: cancer pain, neuralgia, arthralgia) — excluding chest pain from angina and pain with a physiological cause such as labour pain; and (2) all post-operative patients, regardless of symptom presentation. A validated, age-appropriate tool is used. The assessment covers five mandatory content elements: intensity (using an age-appropriate validated pain rating scale),

		character, frequency, location, and duration and any referral or radiation. The assessment is done objectively to allow reliable reassessment comparison.
COP.17.d	Commitment	Section 5.4 addresses pain alleviation initiated based on assessment and underlying condition; patient monitored for response; measures and medication adjusted based on that response. Pain management spans four technique categories per the WHO pain ladder scale: non-pharmacological, medical, surgical, and anaesthetic.

12. Required Records/Evidence Checklist

Written guidance (COP.17.a)

- Written guidance covering: pain screening process, detailed assessment mechanism, pain mitigation techniques, and pain monitoring process.

Pain screening (COP.17.b)

- Pain screening records in the medical record, documented by the doctor or nurse, for each patient on admission.

Detailed pain assessment (COP.17.c)

- Detailed assessment records for patients with pain as a predominant or main symptom (trigger 1) and for all post-operative patients (trigger 2).
- Validated pain assessment tool identified and appropriate to patient type (neonate, paediatric, adult, ventilated).
- Assessment records covering all five elements: intensity (validated scale), character, frequency, location, and duration and referral/radiation.

Pain relief — initiation and adjustment (COP.17.d)

- Records of pain alleviation method or medication initiated for each patient assessed.
- Monitoring records showing patient response to pain alleviation.
- Records showing adjustment of measures or medication based on response.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.17.
2. Guidebook interpretation supplied for COP.17.a through COP.17.d.
3. World Health Organization. WHO pain ladder scale — governing framework for pain management technique categories (COP.17.d).

Disclaimer

This policy reorganises the supplied COP.17 objective elements and approved plain-language content into policy format. No formal stop-work section is included — COP.17 is not in the accreditation stop-work proposals. The fifth-vital-sign framing is stated explicitly in the Statement of intent, Section 3, Section 5.2, and the training section. The two detailed-assessment triggers are preserved separately and kept distinct: (1) pain as predominant or main symptom, and (2) all post-operative patients regardless of symptom presentation. The two scope exclusions from the Guidebook — chest pain from angina and pain with a physiological cause such as labour pain — are preserved in Section 5.3 and the Non-negotiable rules. The five assessment content elements (intensity using age-appropriate validated scale, character, frequency, location, duration and referral/radiation) are preserved exactly. The four WHO pain ladder technique categories (non-pharmacological, medical, surgical, anaesthetic) are all present in Section 5.4, the traceability table, and the training section.